

SmartCare v0.2 - Requirements Specification Template

1. Problem and Scope

SmartCare uses spreadsheets and paper records, which has lead to duplicate bookings and Making It hard to find patient information. It has also led to inconsistent appointment status and limited appointment history. Creating a new version of SmartCare, it provides a small maintainable system for a small clinic that manages patient practitioners and appointments.

In Scope

- Patient information and history lookup
- Practitioner and Patient management
- Creating and viewing appointments
- Updating appointments and cancellations

Out of Scope

- A complex hospital system
- Facial recognition login
- AI treatment recommendations

Requiring validation

- Practitioners directly using the system to view their schedules

2. Stakeholders

Stakeholder	Need	Evidence
Receptionist/Clinic staff	Easy appointment and information to manage	Client Brief: Reports of duplicate bookings
Practitioner	Accurate appointments	Has to support practitioner and appointment management
Patient	Accurate information records.	Currently difficult for staff to locate
Clinic manager	Need a small maintainable system.	Client Brief: Management wants a small software system

3. Functional Requirements

FR-01: The system has to let staff add a patient record

FR-02: Allow staff to view and search patient information

FR-03: Allow staff to add and manage practitioner records

FR-04: Allow staff to create an appointment and link that to the practitioner and patient

FR-05: Record date and time for every appointment

FR-06: System has to display recorded appointments

FR-07: Prevent an appointment being made when patient information and/or practitioner name and/or appointment time is missing

FR-08: Allow appointment details to be updated

FR-09: Allow appointment status to be recorded

FR-10: Store appointment records so appointment history can still be viewed

4. Non-Functional Requirements

NFR-01: The system has to be able to record valid patient, practitioner, and appointment information during normal use.

NFR-02: The system should be maintainable by keeping the program structure simple.

NFR-03: The system should be usable for. Clinic staff. Through understandable output messages.

NFR-04: The system should be easy to test.

NFR-05: The system should be able to save appointment information without losing the history.

5. User Stories

US-01: As a staff member I want to enter a patient or practitioner name and appointment time so that it contains all required information.

US-02: As a staff member I want to view recorded appointments view so that I can see the appointment information.

US-03: As a staff member I want to create an appointment so that all the patient's visits are recorded and accessible.

US-04: As a staff member I want to update an appointment so that the appointment information is accurate and up to date.

US-05: As a staff member I want to view appointment history so that the previous visits can be reviewed.

6. Acceptance Criteria

1. GIVEN A staff member created a patient name, practitioner name and appointment time.

WHEN They book the appointment.

THEN The system records the appointment.

GIVEN Staff member is booking an appointment.

WHEN The patient's name is empty.

THEN The system rejects the booking as the patient's name is empty. The system displays an arrow and makes them fill in the name

GIVEN Appointments have been made.

WHEN The staff member chooses to display the appointments.

THEN The system displays each appointment with its patient, practitioner and time.

7. Assumptions and Open Questions

Assumptions

- Clinic staff will test the first version to manage and record appointments.
- Patient, practitioner, and appointment management are the main features.
- The prototype provides evidence for booking appointments and displaying appointments.as well as making sure all required fields are filled.

Open Questions

- How should each patient be uniquely identified?
- How should double booking be prevented?
- How long should appointment history be kept?
- Should practitioners be allowed to access their own schedules?

8. AI Requirements Review Record

AI suggestion	Evidence?	Decision	Reason	Verification
SMS reminders.	No	Rejected	Not mentioned	No evidence
Facial recognition login.	No	Rejected	Outside small system scope	No evidence
Receptionists create appointments.	Question/assumption	Unverified	Not confirmed	Not explicitly stated
Online payment.	No	Rejected	Not part of the stated scope	No evidence
Practitioners view schedules.	Question/assumption	Unverified	No explicitly stated	Not stated explicitly
AI recommends treatment.	No	Rejected	Outside the stated purpose of the system	No evidence
Cancelled appointments remain in history.	Ask the client to confirm	Confirmed	Should be confirmed by client	Requirement example

9. Reflection

In 150-250 words: What did AI notice that you missed? What did AI invent or overreach on? Which requirement changed after review? Why must requirements have evidence?

AI helped me see what I have missed when writing the requirements. It showed that some requirements very vague and it made me think more about who would be using the system and how the appointments should be managed.

Some of the suggested from AI were not supported from the case study like SMS reminders or facial recognition. These features would make the system way more complicated which is what the client asked not to happen. I rejected these suggestions because there was no evidence is backing them up.

The appointment history requirement changed after review as I clarified that appointment records including cancelling should be kept so they can be viewed later in history. How long the history should be kept for is still not confirmed.

All requirements need evidence as features should be necessary and not built just because. Evidence from the client, stakeholders or the existing system helps make sure the requirements are needed. It keeps the system within task and within scope.